جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فتحيه شعبان رجب ابراهيم
المحافظة : الفيوم **الحي / المدينة :** سنورس **العنوان :** ممم
رقم البطاقة : 28306282100921 **رقم التليفون :** 01061243936 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 28-06-1983 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني + زولاديكس [14165] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فتحيه شعبان رجب ابراهيم
National ID: 28306282100921

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

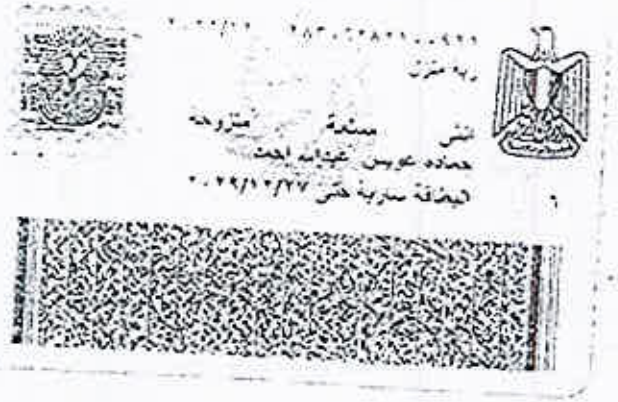
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Second Line Hormonal Treatment Plus Goserline

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





JS7918901

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

٣٨١٤٣

Patient Name	: فتحية شعبان رجب ابراهيم	**Patient Id**	: 10758
Patient Info.	: Female 43Y	**VisitId**	: 100133
Department	: Out Patient	**Collection Date**	: 01/01/2026 13:43:17
Profile NO.	: 4654-2021/ 28306282100921	**Reporting Date**	: 01/01/2026 13:58:54

Clinical Chemistry Report

Test		Result	Unit	Reference Range
Kidney Function				
Calcium Total	:	9.59	mg/dL	8.46 - 10.1

Signature
Dr. Sahar Mansour

Lab Test
[[DOCTYPE:LAB_TEST]]

Name: فتحية شعبان رجب ابراهيم **MR. :** 4481	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient has been receiving Tamoxifen and Zoladex since 2022 following surgery, with the last Zoladex injection administered on 1/1/2026.

The patient is being considered for addition of Zometa due to prolonged Zoladex therapy (3 years) and the presence of bony aches; a dental evaluation is scheduled as a first step.

Radiotherapy to the MRM (PT2 PN2 M0) was discussed in the 6/2022 MDT, with the plan to continue Tamoxifen and Zoladex; the patient is currently on Tamoxifen and Zoladex.

The patient was seen on 1/6/2026 for initiation of Femara (letrozole) in addition to Zoladex, with the last Zoladex dose given on 1/1/2026.

The patient was seen on 29/8/2026 for Femara and Zoladex for 3 months as a baseline symptom management; a CT of the chest, abdomen, and pelvis was ordered as an annual follow-up.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : ندى حسن صلاح كامل

يعتمد ,

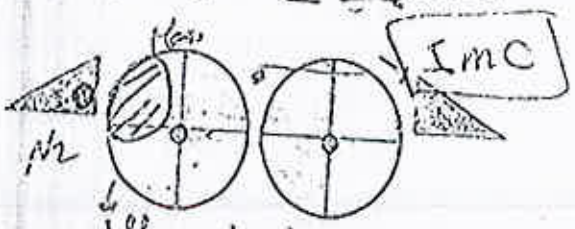
مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

أسست المصرية
هدى
مصر

MDT FORM

Name	فتحية حسن
Age	٤٨ سنة
MRN	٤٦٥٤٨٤
ID	٢٨٢٠٦٢٨٥١٩٩٤
Co morbidity	١٠١٠٢٠٩

١٠١١٤٨٢٢٦



Clinical Data

Bx: IDC GII

IHC:
ER: ٧١٣
PR: ٦١٧
Her1: ٠
Ki67: ٢٥٪

LB

SM: Right UOQ (2cm from nipple) 11X12cm + non mass + skin thickening + intraductal extension + Pathological LNs

Mets Workup: CT CAP → Free

cT4b N3b M0 (cr)
(Inflammatory)

COMMENT:
- Bonescan
- GCHC
- Labs

MDT OUTCOME & PLAN

Mastectomy (while await G bone scan) → start NAC (FEC x3 → Taxotere x3) + Herceptin (with start of RT)
→ Clinical response

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

Patient Name	: الأستاذة / فتحيه شعبان رجب ابراهيم	Request No.	: 134735
Patient No.	: 28306282100921	Request Date	: 06/11/2021
Path No.	: SR-586/21	Verification Date	: 10/11/2021
Sex	: Female	Financial Category	: اورام دار السلام - فئة الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Docto:		IHC No.	:

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass

Nature of the speciment:
Slide revision

Gross Examination:
One block labeled by patient name.

Microscopic examination:
- Histological type: Invasive duct carcinoma
- Grade II
- Angioinvasion: No
- Ductal carcinoma in situ: No
- Lymph node involvement: N/A
- Resection Margins: N/A
- Histologic response to therapy: N/A

Diagnosis:
Right breast mass; Slide revision:
Invasive duct carcinoma grade II
IHC:
ER: Strong nuclear reaction in more than 70% of tumour cells. Positive, score (8/8)
PR: Strong nuclear reaction in about 50% of tumour cells. Positive, score (6/8)
Her-2: No membrane reaction in tumour cells. Negative, score 0
Ki67: 25%

Prof. Dr. Gamal Abd-El Aty
Dr. Abdulrahman Ibrahim

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	: الأستاذه / فتحيه شعبان رجب ابراهيم	Request No.	: 13003692
Patient No.	: 28306282100921	Request Date	: 16/05/2022
Path No.	: 1276/2022	Verification Date	: 22/05/2022
Sex	: Female	Financial Category	: ارام دار السلام - تغطية الدولة
Age	: 38 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Diagnosis:
Rt breast cancer, Rt MRM:
- Invasive Ductal Carcinoma GII with minimal Ductal carcinoma in situ
- Actual tumour cellularity (35%)= 2.1cm
- Free nipple and areola
- Positive (7/16) LNs with focal extracapsular extension, largest metastasis size= 0.7cm
- Free deep margin
- Pathological Partial Response
- Stage: ypT2, ypN2

Prof. Lubna Omar
Dr. Menna Atef

Verified by :
dr.menna atef

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

...neral organization for teaching hospitals
...ittutes
... Maher Teaching Hospital
...logy Department

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى أحمد ماهر التعليمي
قسم الأورام

To whom it may concern

Mrs /fathia shabaan ragab,39 years old female patient presented with RT breast Cancer .

she received neoadjuvant chemotherapy

Pt underwent rt MRM

She Was referred to us for PORT

Dose : 4005 cGy/15f/3w .

Started :8/8/2022.

Ended : 8/9/2022.

Head of department

Dr Hany William

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

وزارة الصحة

NASSER INSTITUTE
ONCOLOGY CENTER
NUCLEAR MEDICINE UNIT

Nasser Institute ...

مركز الأورام

وحدة الطب النووي

Patient Name: Fathya Shaban

Date: 24/2/2022

Tc-99m MDP bone scan.

Desired information : skeletal surveying to detect multiple lesions.

Technique:

Delayed Anterior and posterior whole body scan 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP.

Report:

Delayed Static Phase:-

- Degenerative changes are noticed over dorso-lumbar vertebrae.
- No definite hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- Both kidneys are normally seen

Conclusion:

- *The surveyed skeleton is free of osseous deposits.*

Thanks for reference

Nuclear medicine Unit

Dr. Omnia Talaat, MD

Dr.Sherine Mahdy,MD

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Dar El-Salam Caner Center

مستشفى أورام دار السلام

Radiology Department

قسم الأشعة

Name: Salim shaban ragab
Date: 9 / January 2022

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Enlarged in size (18cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic billiary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Not adequately distended (patient not fasting) however no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Enlarged 14cm , homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- The right kidney shows solid hypoechoic lesion with two small cystic areas seen at the middle anterior cortex, the lesion measures 5x2.5cm

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adenexeal : no abnormalities could be seen (IUCD is seen in place)

Opinion:

Hepatosplenomegaly
Right renal solid lesion with cystic areas ?Neoplastic for contrast enhanced CT

Best regards
Dr. Ismaeel Ramadan, MD

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Name : Fathya Shaaban Ragab
Date : 10-2-2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to the previous study dated 4.11.2021 the current study shows:

1. *Chest:*
   - Regressive course regarding the previously noted :
     - Pathologically enlarged right axillary lymph nodes reaching 1.3 x 0.9 cm compared to 2.5 x 2 cm previously.
     - Enlarged right internal mammary lymph nodes measuring 0.5 x 0.4 cm compared to 0.9 x 0.7 cm previously.
   - Still noted middle lobar tiny benign calcific pulmonary nodule measuring 3 mm.
   - Still noted right lower lung lobe (superior segment) tiny subpleural pulmonary nodule measuring 3 mm, likely insignificant.
   - Otherwise, No sizable pulmonary masses, sizable nodules, areas of consolidation or cavitary lesions.
   - No pathological hilar or mediastinal lymph nodes.
   - No evidence of pleural effusion, pleural thickening or masses seen.
   - Normal cardiac size. No pericardial effusion detected.
   - Normal CT appearance of the thoracic aorta and the great vessels.
2. *Abdomen and pelvis:*
   - The liver is enlarged with homogenous CT texture. No focal lesions or contour irregularities.
   - Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - Newly developed multiple right renal cortical based wedge shaped hypo enhancing lesions, the largest of them measures 1.5x 2.5 cm with related posterior thin rim of collection.
   - Normal size, parenchymal thickness & density of left kidney. No stones or back-pressure changes on either sides.
   - Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
   - Underfilled urinary bladder with no stones.
   - Normal appearance of the other pelvic organs.
   - IUCD is seen in place.
   - No abdominal or pelvic lymphadenopathy or collections.
   - No suspicious lytic or sclerotic osseous lesions.

OPINION:

Follow up study showing:

- *Regressive course regarding the previously noted right axillary and internal mammary lymphadenopathy.*
- *Picture suggestive of right renal pyelonephritis for clinical correlation.*
- *Stationary rest of the study.*

Dr. Aya Deabes, Msc.

Dr. Ola Magdy, MD

CT Scan
[[DOCTYPE:CT_SCAN]]

[[DOCTYPE:CT_SCAN]]
CT Scan

Medical No. : 9732
Patient's Name: fathia shaban ragab ebrahim
Date: 2026-09-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

جوستاف روسي انترناشونال مصر

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA underwent right MRM.

I- CHEST:

- Right upper lobe subpleural reticulations likely post radiation sequelae.
- Diffuse mosaic attenuation of both lungs ? small airway disease for clinical correlation.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- IUCD is seen within the uterus.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- L3 vertebral body tiny lytic focus is seen.

Opinion:

- **L3 vertebral body tiny lytic focus.**
- **No pulmonary or hepatic metastatic lesions.**

Dr.sherif Wagdy

[[DOCTYPE:CT_SCAN]]
CT Scan

٤٤٨٦

Medical No. : 9732
Patient's Name: fathia shaban ragab ebrahim
Date: 2026-09-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

جوستاف روسي انترناشونال مصر

Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post right MRM & ALAND, routine follow up.

Findings:

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

Conclusion:-

Left breast (BIRADS 1): normal study.

Clear right mastectomy bed and axilla.

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	